



February 23, 2026

Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health & Human Services
200 Independence Ave, SW
Washington, DC 20201

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
U.S. Department of Health & Human Services
330 C St SW, Floor 7
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Secretary Kennedy and Assistant Secretary Keane:

On behalf of the AI Healthcare Coalition (AIHC), thank you for the opportunity to contribute to the ongoing work by the US Department of Health and Human Services (HHS) Office of the Deputy Secretary, in collaboration with the Assistant Secretary for Technology Policy and Office of the National Coordinator for Health Information Technology (ASTP/ONC), to make artificial intelligence (AI) an essential part of clinical care.

AIHC was founded in 2019 and brings together healthcare AI innovators and stakeholders to advocate for patient access to safe, ethically developed health AI. Our membership includes innovator companies that create cutting edge AI powered health services (or devices) that benefit patients and enhance healthcare opportunities.

As you continue to consider regulatory, reimbursement, and research and development challenges presented by AI innovation, AIHC is pleased to provide these recommendations for your consideration. Additionally, we hope to be a continuing resource to HHS and ASTP/ONC as novel challenges emerge.

Specifically, we recommend HHS:

- Create permanent Medicare payment pathways for AI technologies;
- Implement clear coverage pathways for AI technologies, such as President Trump's; Medicare Coverage for Innovative Technologies (MCIT) program;
- Ensure separate payment for AI services and underlying imaging services;
- Exclude AI Services from the Outpatient Prospective Payment System (OPPS) cap on imaging services;
- Create a pathway for permanent, separate payment for AI services whose NTAP rate has expired; *and*

- Exclude FDA authorized AI systems from HHS/OCR's Sec. 1557 regulations.

We offer further comment on these recommendations below.

I. Create Permanent Medicare Payment Pathways for AI Technologies

While some AI healthcare services are currently reimbursed under the Medicare program, there is no clear pathway for clinicians and innovators to obtain reimbursement for AI services. As more AI services are rigorously validated and FDA-authorized, a clear reimbursement pathway— or multiple pathways— is/are needed to ensure that Medicare beneficiaries and clinicians can access these services. Notably, AI services are diverse and varied, spanning multiple specialties and incorporating modalities that vary in their use of algorithmic design and machine learning.

For example, in the case of AI services provided in the outpatient setting, we often see such services assigned to Clinical Ambulatory Payment Classification (APC) that does not adequately reimburse for provider costs because there is no specific pathway for AI services. While CMS does have a New Technology APC process to allow reimbursement for new technologies in the outpatient setting, that process was not created to facilitate the payment of AI technologies, and often presents too short or low a payment to allow access. For these reasons, we support a 5-year outpatient payment rate for AI technologies. Permanent payment pathways will create certainty for AI technology developers and providers, which would drive investment and accelerate innovation.

II. Implement Clear Coverage Pathways for AI Technologies, such as President Trump's Medicare Coverage for Innovative Technologies (MCIT) Program¹

During President Trump's first term, the MCIT program was improved to provide a transitional Medicare coverage policy for certain medical technologies that receive "breakthrough technology" status under the FDA. President Biden pulled back this policy and instead finalized a similar policy entitled the "Transitional Coverage for Emerging Technologies," or TCET, program. While this program makes some positive steps forward by requiring Centers for Medicare & Medicaid Services (CMS) and the U.S. Food & Drug Administration (FDA) to work contemporaneously and iteratively with innovator applicant companies, the agencies have said that very few — approximately four— applications would be eligible for the TCET process annually. This is far too few. We urge the Administration to facilitate timely coverage for AI health technologies by finalizing an MCIT -like policy. In addition to increased patient access, such a policy would create confidence for investment in these important technological advancements, aiding commercialization.

¹ Department of Health & Human Services, Centers for Medicare & Medicaid Services. 42 CFR Part 405 [CMS-3372-IFC] RIN 0938-AT88. Medicare program; Medicare Coverage of Innovative technology (MCIT) and Definition of "Reasonable and Necessary"; Delay of Effective Date; Public Comment Period (March 17, 2021)

III. Ensure Separate Payment for AI Services and Underlying Imaging Services

CMS recently finalized a policy in its Outpatient Prospective Payment System (OPPS) rule to allow separate payment for the AI service and the underlying imaging service,² but we see ongoing challenges in specialty areas where imaging services and payment are bundled (for example, AI that is viewed as computer-aided detection (CAD)). We urge the administration to provide further guidance in this area to ensure that its separate payment policy, as finalized by CMS in the CY 2023 OPPS final rule, is effectuated.

IV. Exclude AI Services from the OPPS Cap on Imaging Services

Sec. 5102(b) of the Deficit Reduction Act (DRA) requires that “for imaging services [...] furnished on or after January 1, 2007, [CMS] will cap the [technical component] TC of the PFS payment amount for the year (prior to geographic adjustment) by the [annual] OPPS payment amount (prior to geographic adjustment). Congress enacted 5102(b) also known as the OPPS cap, to address rapid growth in spending on imaging services, particularly for advanced imaging modalities, such as computed tomography (CT), magnetic resonance imaging (MRI), and nuclear medicine, as compared to the growth in spending among less advanced imaging modalities, such as x-ray or ultrasound.

As is clear in the statutory text, the DRA envisioned services such as x-rays, ultrasound, PET, MRI, and CT as such “imaging services,” not autonomous AI other Software-as-a-Service (SaaS) technologies. Further, when CMS implemented the OPPS cap pursuant to Sec. 5102(b), CMS expressly excluded some services because those services encompassed more than the use of imaging technology alone. CMS should recognize an additional exclusion for service for TC-only services for which there is no associated PC, as CMS has previously recognized that codes without a TC / PC split should not be listed on the OPPS Cap List.³

V. Create a Pathway for Permanent, Separate Payment for AI Services whose NTAP Rate has Expired

Like CMS’ New Tech APC Program, inpatient AI services may apply for and receive a New Technology Add-on Payment (NTAP) payment for 2-3 years if they meet certain criteria. Once NTAP expires, CMS has reasoned that the cost of the technology should be covered by the DRG payment. The industry

² Department of Health and Human Services, Centers for Medicare & Medicaid Services. 42 CFR Parts 405, 410, 411, 412, 413, 416, 419, 424, 485, and 489 [CMS 1772-FC; CMS 1744-F; CMS 3419-F; CMS 5531-F; CMS 9912-F] RIN 0938-AU82. Medicare Program: Hospital Outpatient Prospective Payment and Ambulatory Surgical Center Payment Systems and Quality Reporting Programs; Organ Acquisition; Rural Emergency Hospitals: Payment Policies, Conditions of Participation. Provider Enrollment, Physician Self-Referral; New Service Category for Hospital Outpatient Department Prior Authorization Process; Overall Hospital Quality Star Rating; COVID-19. (November 23, 2022).

³ Department of Health and Human Services, Centers for Medicare & Medicaid Services. 42 CFR Parts 405, 410, 414, 424, 425, 427, 428, 495, and 512 [CMS-1832-P] RIN 0938-AV50. Medicare and Medicaid Programs; CY 2026 Payment Policies Under the Physician Fee Schedule and Other Changes to Part B Payment and Coverage Policies; Medicare Shared Savings Program Requirements; and Medicare Prescription Drug Inflation Rebate Program. (July 16, 2025).

does not have data as to whether this is happening, and we believe that a lack of ongoing separate payment leads to a lack of provider adoption / patient access. This is particularly true given recent CMS proposals that may reduce the time NTAP status applies to a new technology in practice. The ambiguity about reimbursement post-NTAP disincentivizes investment in these tools which can benefit the Medicare program. If appropriately reimbursed, such tools are likely to be adopted by healthcare providers, improve patient care, and create efficiencies.

VI. Exclude FDA Authorized AI systems from HHS/OCR's Sec. 1557 Regulations

On May 6, 2024, HHS / OCR and CMS finalized updates to regulations implementing section 1557 of the Affordable Care Act (ACA) to increase anti-discrimination protections for covered health care entities.⁴ Among other provisions, this rule established new obligations for providers who utilize AI and other patient care decision support tools. At the time of issuance, HHS noted this regulation was in furtherance of President Biden's now-rescinded EO on "Safe, Secure, and Trustworthy Development and Use of Artificial Intelligence."

Specifically, the Sec. 1557 final rule applies nondiscrimination principles to the use of new and broadly defined "patient care decision support tools" in clinical care and requires those subject to the rule to identify and mitigate discrimination when they use patient decision support tools, including automated and non-automated tools, mechanisms, methods, and technology to provide patient care. As of now, covered entities are required to comply with the new patient care decision support tool requirements, including ongoing monitoring and risk mitigation responsibilities.

Although stakeholders expressed concerns regarding regulatory duplication at the time that the rule was proposed, OCR expressly declined to exclude FDA-authorized technologies from the scope of the rule. Importantly, while ONC's requirements for predictive DSIs apply to health information technology developers, Section 1557's requirements apply to covered entity users of patient care support tools. These requirements create a redundant regulatory burden for AI innovator companies and healthcare providers, especially for those tools that are already subject to FDA review and authorization.

Conclusion

Thank you for the opportunity to provide comments on HHS and ASTP/ONC's work to improve patient access to AI health services. AI health innovation has the potential to transform the clinical care delivery system, and you are uniquely positioned to usher in this new era.

We look forward to working with you to ensure patients have access to advanced technologies that improve clinical care and health outcomes. If you have any questions about our comments, please contact me at cybil.roehrenbeck@ai-coalition.org.

Kind regards,

⁴ "Nondiscrimination in Health Programs and Activities," 89 Fed. Reg. 37522 (May 6, 2024)



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